Pharmacy prep Exam Case # 3

You are working in a pharmacy. A Registered nurse is waiting in the pharmacy to ask you a clarification on a written prescription by her doctor for one of his patients.

- Do not use the blank space beside the prescription for making any rough notes.

If the solution suggested is different from the doctor's prescription please document the alternative otherwise just sign in as- process as prescribed.

The patient record is on the desk/table, for your reference. Document the alternative suggested in the form of a prescription in the sheet of paper provided

Your objective is to:

1. Review the prescription for any drug related problem(s).
2. You may speak with the physician if:
   a. you need more information
   b. you wish to discuss the patient's therapy
3. **You must speak directly to the nurse to:**
   a. identify and explain any drug related problem(s)
   b. recommend an appropriate solution to the problem(s)

Note: You are NOT required to dispense the prescription or counsel the patient.

Time Frame: You have 7 minutes to complete this station

References: CPS, TC, PSC

Patient: Sam Rodrigues

Age: 65 years

Medical condition: COPD & recently having urinary retention problems

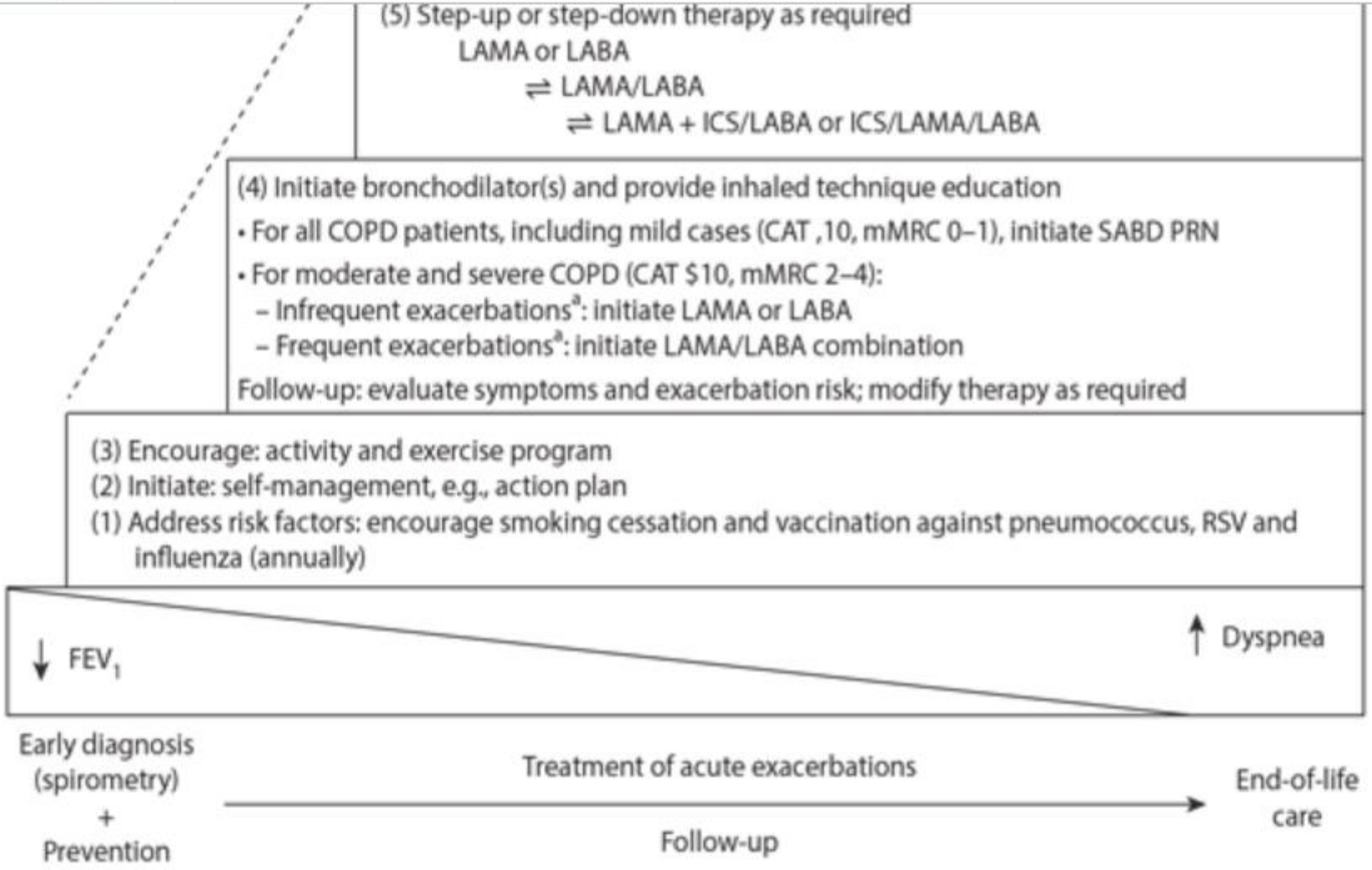
Allergies: Penicillin

Medication	Directions	Qty	Last filled
Ventolin (Salbutamol) HFA	2 puffs qid	200 dose	Using from 3 years ago
Atrovent (Ipratropium bromide)	1 puff qid	200 dose	Using from 3 years ago
Spiriva 18 mcg (tiotropium)	Inhale 1 cap daily via handihaler	90 dose	Using from 1 month ago

- Actor: “ Nurse comes in”
- I have a common patient of ours who is having a lot of breathing problems. And Dr wants to know what would we give to this patient for his COPD ?
- We want to reduce hospitalizations and improve lung functions.
- Only if the student asks: >1 exacerbation in two consecutive years

Answers ref: CTC

- Stop **Atrovent (ipratropium) because** of urinary symptoms
- Addition of a combined **LABA TO TIOTROPIUM** for patients with moderate to severe COPD & repeated exacerbations (>= 2 requiring antibiotics/steroids or >1 exacerbation requiring hospitalization).
- Recommend to add:
- *Tiotropium/olodaterol (*Inspiolto Respimat): SMI 2.5/2.5 mcg per actuation: 2 actuations inhaled once daily
- *umeclidinium/vilanterol*Anoro Ellipta: DPI 62.5/25 mcg per actuation: 1 actuation inhaled once daily.
- Ask nurse which device she prefers?
- Reduces frequency & severity of exacerbations & improves health & reduce hospitalizations



[a] A patient with frequent exacerbations is classified as: ≥2 exacerbations requiring antibiotics and/or systemic corticosteroids *or* ≥1 exacerbation requiring hospitalization in the past year.